Three Mistakes Families Make When Keeping an Eye on How Dad Is Managing

What each one looks like, why it happens, and the small habit that replaces it.

Most families keep track of how a parent is doing the same way: from memory, a little at a time. It's the natural way, and none of it is carelessness. Three small habits improve on it.

1 Relying on memory to compare

WHAT IT LOOKS LIKE Someone asks whether Dad is slower getting out of his chair than he was last year, and everyone at the table has a different answer. Your brother says he's always pushed up off the arms like that. You're not so sure.

WHY IT HAPPENS Nothing was wrong last year, so nobody wrote anything down. That was reasonable. But how much help a person needs often changes over time, and a change that comes slowly leaves no date behind unless someone gives it one. [1]

> **WHAT TO DO INSTEAD** Describe one ordinary week and put the date at the top: what Dad does on his own, in plain sentences, with “not noted” wherever you didn't see. Keep it where you'll find it again. The next time the question comes up, you'll be comparing two pages.

2 Overlooking gradual increases in family help

WHAT IT LOOKS LIKE “Dad does everything himself.” And he very nearly does. Your sister began driving him to appointments two winters ago, you took over the online bills, and a neighbor brings his bins in.

WHY IT HAPPENS Each thing was small when it started, and nobody counts small things. AARP's planning guide describes the pattern: caregiving may start with driving a loved one to get groceries or to the doctor, and later include preparing meals or handling bills. [2]

> **WHAT TO DO INSTEAD** List everything anyone does for him and, beside each, roughly when it began: always, in the last year or two, or not sure. Ask one or two of the helpers whether the list matches what they actually do. It isn't a verdict on Dad. It's part of how things are today.

3 Drawing clinical conclusions from isolated observations

WHAT IT LOOKS LIKE Dad forgets one appointment. By the weekend, “he forgot Thursday” has become “his memory is going” in the family group text.

WHY IT HAPPENS One clear event is easier to hold on to than fifty ordinary days, and worry fills in the rest. The people who assess for a living are asked to look wider: the form home health clinicians complete tells them to record a person's usual performance. [3]

> **WHAT TO DO INSTEAD** Split the note in two. On one side, what happened, with the date: “Missed the dentist, Thursday.” On the other, what you made of it. Keep the first. Bring it with the rest of your notes to his doctor, and let what it means be theirs to say.

One habit covers all three

Write down what happened, with the date and who saw it. Compare notes with notes. What they mean is for the professionals who know Dad.

What this guide doesn't do

It doesn't say whether anything about Dad has changed, what a change would mean, or when he should be looked at again. It names three ordinary habits and a replacement for each. They are the three the Family Care Roadmap names for this step.

Ready for the next step?

You know the three habits: a dated page, a list of who helps, and notes kept apart from conclusions. The care interview gives them a structure: it asks about each area of Dad's daily life in turn, records each answer with its source and date, and turns it all into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

The line about needs changing over time is the Administration for Community Living's, the description of how family help grows is AARP's, and “usual performance” comes from the assessment form home health clinicians complete. The three mistakes are the Family Care Roadmap's own.

1. Administration for Community Living, How Much Care Will You Need?
2. AARP, Prepare to Care: A Planning Guide for Families
3. Centers for Medicare & Medicaid Services, Outcome and Assessment Information Set Version E1 (OASIS-E1), All Items

From the Family Care Roadmap · Richard Ladson, Certified Medicaid Planner. Draft for review, generated 2026-10-04.